National Cancer Control Programme (NCCP) 00857 · clinician draft

# Pembrolizumab 200 mg + carboplatin AUC 5 / paclitaxel → AC

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Phase 1 · Day 1 carboplatin combination

**Active medicines:** Carboplatin + Paclitaxel + Pembrolizumab on its separate schedule

**Emetogenic risk:** High · carboplatin AUC ≥4

| Medicine | Dose / route | When |
|---|---|---|
| Aprepitant | 125 mg orally | Day 1 |
| Ondansetron | 16 mg orally | Day 1<br>Individual dose / QT / hepatic review |
| Dexamethasone | 12 mg orally | Day 1<br>Reconcile chemotherapy premedication steroids |
| Aprepitant | 80 mg orally daily | Days 2–3 |
| Metoclopramide | 10 mg orally up to three times daily | Subsequent days: duration and use to be confirmed<br>Individual contraindications, renal function and duration review |
| Dexamethasone | 8 mg orally daily | Days 2–4<br>Carboplatin AUC ≥4 antiemetic row; reconcile total steroid exposure |

### Additional phase-specific support

- First paclitaxel dose: dexamethasone 8 mg intravenous (IV), chlorphenamine 10 mg IV and famotidine 20 mg IV 30 minutes before treatment.
- Day 8 and later: no routine dexamethasone unless the consultant specifies it for reaction/risk; chlorphenamine continues and any famotidine omission requires consultant/local-policy agreement.
- Reconcile antiemetic and taxane dexamethasone into one prescription; never add duplicate steroid courses.
- Four 21-day cycles: paclitaxel on Days 1, 8 and 15; carboplatin AUC 5 on Day 1. Pembrolizumab 200 mg on Day 1 of each cycle (every three weeks).

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00857 v3 · p9 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-20 · SACTCheck v0.84.13 · Regimen-level drafts; prescriber and oncology-pharmacy verification required